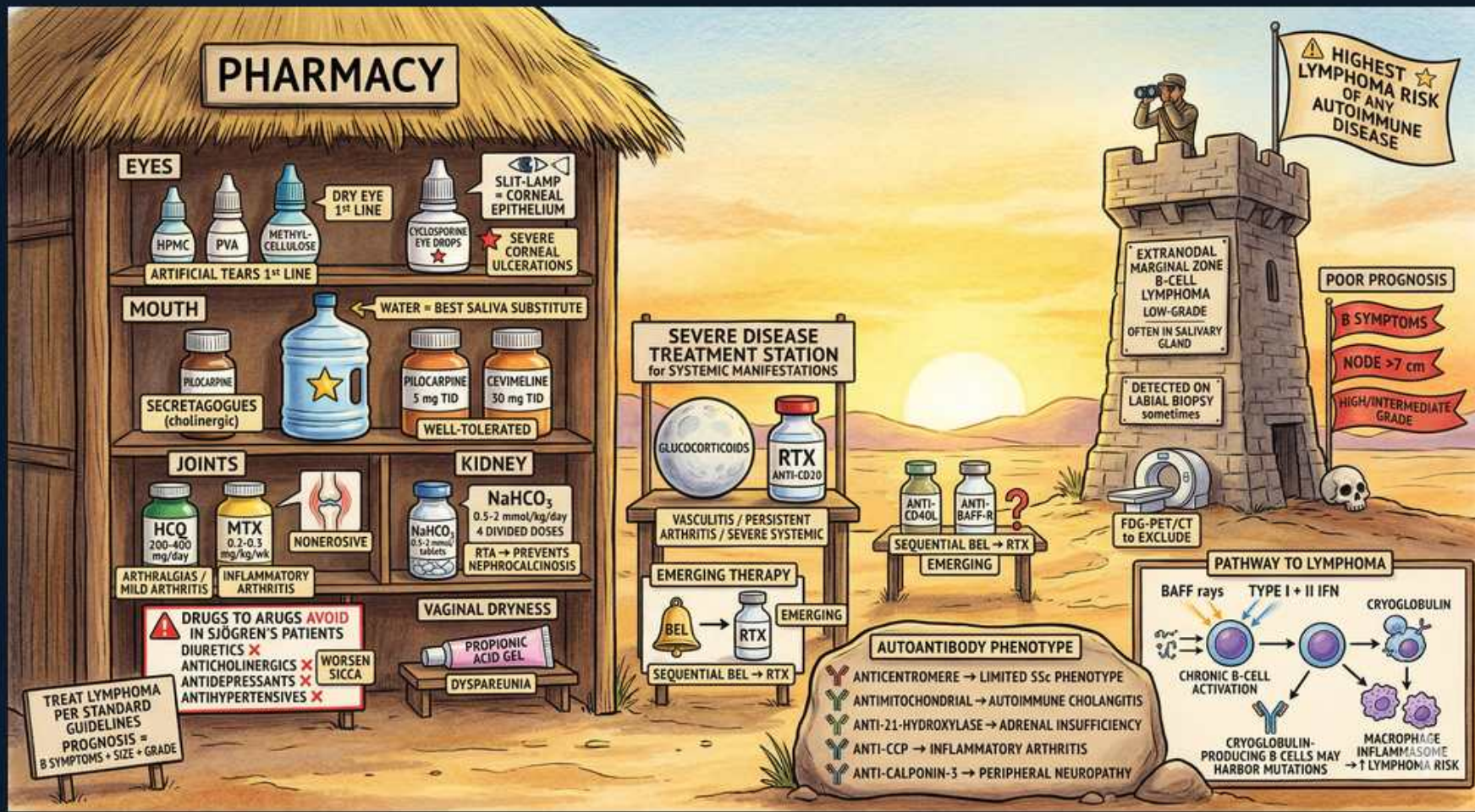


Sjögren — Complications, Lymphoma & Treatment



1. Highest lymphoma risk

WHAT YOU SEE

Watchtower banner: highest lymphoma risk of any autoimmune disease

WHAT IT ENCODES

Sjögren syndrome carries the HIGHEST lymphoma risk of any autoimmune disease, roughly a 5-10% lifetime risk and a ~15-44x relative risk versus the general population. Chronic antigen-driven B-cell proliferation in inflamed glandular tissue is the mechanism. Risk is concentrated in patients with persistent salivary gland swelling, cryoglobulinemia, low C4, and palpable purpura.

BOARD TRAP

Reassuring a Sjögren patient that their autoimmune disease carries no cancer risk — WRONG. Sjögren has the single highest lymphoma risk among autoimmune diseases. Discriminator: persistent/firm parotid enlargement, low C4, and cryoglobulins are red flags that should prompt biopsy, not watchful reassurance.

2. MALT / marginal zone lymphoma

WHAT YOU SEE

Tower sign: extranodal marginal zone lymphoma, low-grade, often in salivary gland

WHAT IT ENCODES

The classic Sjögren malignancy is extranodal marginal zone (MALT) lymphoma, a LOW-GRADE B-cell non-Hodgkin lymphoma arising most often in the salivary (parotid) glands. It evolves from chronic lymphoepithelial sialadenitis. Localized indolent MALT may be observed or treated locally; transformation to high-grade DLBCL is the feared progression.

BOARD TRAP

Naming Hodgkin lymphoma or DLBCL as the first/most characteristic lymphoma in Sjögren — WRONG. The hallmark is low-grade MALT (marginal zone) lymphoma of the salivary glands. Discriminator: an indolent parotid mass with monoclonal B-cells = MALT; rapidly enlarging mass with B symptoms suggests transformation to high-grade DLBCL.

3. Lymphoma red flags

WHAT YOU SEE

List: B symptoms, node >2cm, persistent parotid swelling

WHAT IT ENCODES

Lymphoma warning signs in Sjögren: persistent or firm/unilateral PAROTID enlargement, lymphadenopathy >2 cm or splenomegaly, B symptoms (fever, night sweats, weight loss), new monoclonal gammopathy, cryoglobulinemia, and a PERSISTENTLY LOW C4. New leukopenia and disappearance of a previously positive rheumatoid factor can also signal clonal evolution.

BOARD TRAP

Attributing recurrent bilateral parotid swelling and a low C4 to a benign sicca flare — WRONG. Persistent/firm parotid swelling plus low C4 and cryoglobulins are the strongest predictors of lymphoma. Discriminator: episodic, soft, symmetric swelling is usually benign sialadenitis; persistent, firm, or unilateral enlargement warrants imaging and biopsy.

4. Cryoglobulinemia → lymphoma

WHAT YOU SEE

Pathway diagram: cryoglobulin, chronic B-cell activation, low C4

WHAT IT ENCODES

Mixed (type II) cryoglobulinemia in Sjögren reflects clonal B-cell expansion producing monoclonal IgM rheumatoid factor; it manifests as palpable purpura, peripheral neuropathy, glomerulonephritis, and consumed complement (low C4). Cryoglobulinemia plus low C4 and purpura is one of the strongest independent predictors of progression to lymphoma.

BOARD TRAP

Dismissing palpable purpura with low complement in a Sjögren patient as incidental — WRONG. This cryoglobulinemic-vasculitis picture marks high lymphoma risk. Discriminator: low C4 + cryoglobulins + purpura = high-risk phenotype requiring close surveillance (and often rituximab), not routine sicca management.

5. Artificial tears 1st line

WHAT YOU SEE

EYES shelf: artificial tears 1st line

WHAT IT ENCODES

Keratoconjunctivitis sicca (dry eyes) is treated first-line with preservative-free artificial tears and lid hygiene; escalate to topical cyclosporine or lifitegrast for moderate-severe ocular inflammation, and punctal plugs to retain tears. Schirmer test <5 mm/5 min and ocular staining confirm aqueous-deficient dry eye.

BOARD TRAP

Jumping to systemic immunosuppression or steroids for isolated dry eyes — WRONG. Sicca symptoms are managed with LOCAL/topical therapy first. Discriminator: reserve systemic agents for extraglandular/systemic disease; dry eyes alone get artificial tears, then topical cyclosporine/lifitegrast, not oral steroids.

6. Pilocarpine secretagogue

WHAT YOU SEE

MOUTH shelf: pilocarpine, secretagogues, well tolerated

WHAT IT ENCODES

Xerostomia (dry mouth) is treated with muscarinic secretagogues — pilocarpine (~5 mg three to four times daily) or cevimeline — which stimulate residual exocrine secretion; sugar-free gum/lozenges and saliva substitutes help. Cholinergic side effects include sweating, flushing, and GI upset.

BOARD TRAP

Prescribing pilocarpine to a Sjögren patient with poorly controlled asthma or narrow-angle glaucoma — WRONG. Muscarinic agonists are contraindicated/cautioned in asthma/COPD, bradycardia, and narrow-angle glaucoma. Discriminator: check for reactive airway disease and angle-closure glaucoma before starting cholinergic secretagogues; use saliva substitutes instead if contraindicated.

7. Hydroxychloroquine for arthralgia

WHAT YOU SEE

JOINTS shelf: HCQ for arthralgia/mild arthritis

WHAT IT ENCODES

Hydroxychloroquine is used for Sjögren arthralgias, fatigue, and mild inflammatory arthritis, and may help cutaneous manifestations. Baseline and annual ophthalmologic screening is required to monitor for retinal toxicity. It does not reliably improve the sicca symptoms themselves.

BOARD TRAP

Expecting hydroxychloroquine to fix the dry eyes and dry mouth — WRONG. HCQ addresses arthralgia/fatigue, not the sicca complex. Discriminator: treat sicca with local secretagogues/artificial tears; use HCQ for the musculoskeletal/constitutional symptoms, with mandatory retinal monitoring.

8. NaHCO₃ for RTA

WHAT YOU SEE

KIDNEY shelf: NaHCO₃, bicarb for distal RTA, nephrocalcinosis

WHAT IT ENCODES

The classic renal lesion is tubulointerstitial nephritis causing distal (type 1) renal tubular acidosis — a non-anion-gap metabolic acidosis with inappropriately alkaline urine (pH >5.5), hypokalemia, and risk of nephrocalcinosis and calcium phosphate stones. Treat with oral bicarbonate (and potassium) to correct acidosis and prevent stones; hypokalemia can cause periodic paralysis.

BOARD TRAP

Labeling the metabolic acidosis in a Sjögren patient as a high-anion-gap process or proximal (type 2) RTA — WRONG. Sjögren causes DISTAL (type 1) RTA with hypokalemia and urine pH >5.5. Discriminator: distal RTA fails to acidify urine (pH stays high despite acidemia) and predisposes to nephrocalcinosis, treated with bicarbonate replacement.

9. Rituximab for severe systemic

WHAT YOU SEE

Severe disease station: RTX anti-CD20 for vasculitis/persistent arthritis

WHAT IT ENCODES

Rituximab (anti-CD20 B-cell depletion) is used for SEVERE systemic Sjögren — cryoglobulinemic vasculitis, severe parotid swelling, persistent inflammatory arthritis, and some neurologic/pulmonary involvement. It targets the pathogenic B-cell compartment and is part of management when MALT lymphoma develops (e.g., R-based regimens).

BOARD TRAP

Reaching for rituximab to relieve isolated dry eyes/mouth — WRONG. Rituximab is reserved for severe systemic/extraglandular disease, not sicca alone. Discriminator: sicca = local therapy; rituximab = vasculitis, cryoglobulinemia, refractory arthritis, or lymphoma.

10. Glucocorticoids for systemic

WHAT YOU SEE

Severe disease treatment station with glucocorticoids

WHAT IT ENCODES

Systemic corticosteroids are reserved for significant extraglandular/systemic manifestations — vasculitis, severe arthritis, pneumonitis, or neurologic disease — generally as short courses or bridges to steroid-sparing agents. They are NOT appropriate for the sicca symptoms, which respond to local measures.

BOARD TRAP

Using systemic steroids for isolated dry eyes and dry mouth — WRONG. Sicca is managed with local/topical therapy (artificial tears, pilocarpine), not systemic steroids. Discriminator: limit steroids to organ-threatening extraglandular disease; chronic steroids for sicca add toxicity without benefit.

11. Avoid drying drugs

WHAT YOU SEE

Warning: avoid diuretics, anticholinergics, antihistamines, antihypertensives

WHAT IT ENCODES

Avoid medications that worsen sicca: anticholinergics, sedating antihistamines, tricyclic antidepressants, diuretics, and certain antihypertensives all reduce secretions. Reviewing and deprescribing offending drugs is a simple, high-yield first step that often improves dry mouth/eyes before adding new therapy.

BOARD TRAP

Starting a sedating antihistamine or anticholinergic for incidental symptoms in a Sjögren patient — WRONG. These dry the patient further and worsen sicca. Discriminator: before escalating sicca therapy, scrub the medication list for anticholinergic/antihistamine/diuretic culprits and substitute non-drying alternatives.

12. Anti-centromere → limited SSc phenotype

WHAT YOU SEE

Autoantibody phenotype list: anticentromere, anti-CCP, anti-mitochondrial

WHAT IT ENCODES

Sjögren overlaps with other autoimmune diseases, and overlap antibodies predict phenotype: anti-centromere → limited SSc features (and higher PAH risk), anti-CCP → erosive inflammatory arthritis (RA overlap), anti-mitochondrial antibody (AMA) → primary biliary cholangitis/autoimmune cholangitis, anti-calponin → peripheral neuropathy. The core Sjögren antibodies remain anti-Ro/SSA and anti-La/SSB.

BOARD TRAP

Forgetting that anti-Ro/SSA and anti-La/SSB are the defining Sjögren antibodies and over-weighting an overlap antibody — WRONG. SSA/SSB (with focal lymphocytic sialadenitis on minor salivary gland biopsy) define Sjögren; overlap antibodies just predict additional phenotypes. Discriminator: anti-Ro/SSA in a pregnant patient also carries the risk of neonatal lupus and congenital complete heart block in the fetus.

13. Vaginal dryness / dyspareunia

WHAT YOU SEE

Vaginal dryness shelf: dyspareunia, vaginal gels

WHAT IT ENCODES

The exocrine/mucosal dryness of Sjögren extends beyond eyes and mouth to vaginal dryness, causing dyspareunia and increased risk of vaginitis. Manage with vaginal moisturizers/lubricants and, when appropriate, local estrogen — analogous to the local-therapy-first principle used for ocular and oral sicca.

BOARD TRAP

Treating mucosal/genital dryness with systemic immunosuppression — WRONG. Like ocular and oral sicca, vaginal dryness is managed with LOCAL measures (lubricants, moisturizers, topical estrogen). Discriminator: systemic therapy is reserved for systemic disease; glandular/mucosal dryness is a topical-therapy problem.

14. Treat lymphoma per oncology

WHAT YOU SEE

Note: treat lymphoma per standard guidelines, prognosis by stage/grade

WHAT IT ENCODES

Once lymphoma develops it is treated per standard oncologic protocols stratified by stage and grade: localized indolent MALT may be observed or treated locally (e.g., low-dose radiation), while disseminated or high-grade (DLBCL) disease needs systemic immunochemotherapy (rituximab-based regimens). Low-grade MALT generally carries a favorable prognosis.

BOARD TRAP

Assuming all Sjögren-associated lymphomas need aggressive chemotherapy — WRONG. Indolent localized MALT may be watched or treated locally, whereas high-grade/disseminated disease needs systemic therapy. Discriminator: management is driven by stage and grade — match treatment intensity to the histology rather than treating every lymphoma the same way.